NHS
Royal Devon University Healthcare
NHS Foundation Trust

TEG6s Protocol

For postpartum haemorrhage

Blood loss >1000ml (Measured or suspected) Or **Clinical Concerns/abnormal vital signs** e.g Abruption or concealed bleeding BP <90/40, HR >120 Sats <92%, RR >30

Take Bloods **POC:** TEG6, lactate, Hb **Labs:** FBC, coag +clauss fibrinogen X match if needed

Optimise Patient
Temp >36 HB>80g/L pH >7.2 ionised Ca2+ >1mmol/L

Review other causes of bleeding

Give Tranexamic Acid (if not already)

ONGOING BLEEDING or CLINICAL CONCERN

NO
No blood products required

Clinical Monitoring

Any of the following?
- Clinical concern
- Suspected further bleeding (including concealed)
- >500mls further blood loss

YES

Only transfuse blood products if patient is bleeding

1

Review CFF amplitude at 10 minutes (CFF A10)

CFF MA (max amplitude) can be used if occurs before 10 mins)

If CFF A10 >17mm or fibrinogen >2g/L — **No fibrinogen replacement**

CFF A10 < 17mm or fibrinogen <2 g/L → **Give fibrinogen concentrate** CFF A10 10-17mm =4g* CFF A10 <10mm =6g

**If no fibrinogen available give 2x cryoprecipitate*

Any of the following?
- Any blood products given
- >500mls further blood loss
- Clinical concern

Ensure fibrinogen corrected (CFF A10 >17mm and/or fibrinogen >2g/L) prior to further TEG interpretations

2

Review CK-R time

If CK-R <7.6 mins or normal PT/APTT — **No FFP required**

CK-R >7.6 mins Or prolonged PT/APTT → **Give FFP** Booking weight <50Kg = 3 units >50Kg +4units

3

Review FBC

Platelets >75 — **No platelets required**

Platelets <75 → **Give platelets** 1 adult unit

Cautions/limitations
- Inherited Bleeding disorders (including von Willebrand disease)
- LMWH, warfarin, aspirin, clopidogrel, direct oral anticoagulants

These cases should be managed jointly with haematology

If any concerns discuss with haematologist

Useful Numbers:
Haem. reg. bleep 879
Blood Bank 2467/2466 (OOH bleep 227)
Haematology Lab 2918 (OOH bleep 227)

Adapted from CYMRU NHS Wales